

GENITAL ULCER SYNDROME

First assess and advise the patient with genital ulcer/s ➔ 49. The patient may have a blister, sore or an ulcer.

First treat for **herpes**:

- Stress importance of condoms as herpes is a lifelong infection and transmission can occur even when no sores. HIV transmission risk increases when there are ulcers/sores.
- Advise to keep lesions clean and dry.
- If pain, give **ibuprofen** 400mg 8 hourly with food for up to 5 days (avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease).
- Give **aciclovir** 400mg 8 hourly for 7 days if any of:
 - HIV positive or HIV unknown.
 - Pregnant (if patient ≥ 28 weeks pregnant, consider risk of neonatal herpes, refer).
 - Not sexually active in the last 3 months.
- If recurrent ulcers, refer for laboratory testing. If ≥ 4 episodes of laboratory-confirmed herpes simplex in 1 year, refer for ongoing suppressive therapy.



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If patient sexually active in the past 3 months, also treat for **genital ulcer syndrome (GUS)** below:
Does patient have a vaginal/urethral discharge?

No

Yes

Treat for **GUS**

Treat for **GUS with VDS/MUS**

Pregnant woman

Does patient have severe penicillin allergy³?

Man or non-pregnant woman

Yes

No

Refer for confirmation of diagnosis and possible penicillin desensitisation.

- Give single dose **benzathine benzylpenicillin** 2.4MU IM¹.
- If benzathine benzylpenicillin unavailable, give instead **amoxicillin** 1g 8 hourly and **probenecid** 250mg 8 hourly for 14 days. Advise to return in 6 months for RPR: if positive ➔ 53.
- Give partner notification slip/s with code: GUS.

- Give **doxycycline** 100mg 12 hourly for 14 days.
- Advise to return in 6 months for RPR: if positive ➔ 53.
- Give partner notification slip/s with code: GUS.

- Give single dose **ceftriaxone** 250mg IM² and **azithromycin** 1g orally and **benzathine benzylpenicillin** 2.4MU IM¹.
- If severe penicillin allergy³, omit ceftriaxone, increase **azithromycin** to 2g and give **doxycycline** 100mg 12 hourly for 14 days. If pregnant/breastfeeding, refer instead.
- Advise to return in 6 months for RPR: if positive ➔ 53.
- If patient or partner has vaginal discharge syndrome (VDS), also give single dose **metronidazole**⁴ 2g orally. If patient has discharge that is itchy or curd-like or vulva inflamed (red, swollen or painful), also give single dose **clotrimazole vaginal pessary** 500mg inserted at night.
- Give partner notification slip/s with code: GUS + VDS/MUS.

Does patient also have enlarged, hot, tender lymph node/s in groin?

No

Yes

Review in 7 days

- If no better and patient already received azithromycin, discuss/refer, otherwise give single dose **azithromycin** 1g.
- Advise to return if still no better after 7 days: refer.

Also treat for **bubo**:

- Give **azithromycin** 1g now and repeat **azithromycin** 1g weekly for another 2 weeks.
- If fluctuant lymph node, aspirate pus through healthy skin in sterile manner every 3 days as needed.
- Give partner notification slip/s with code: Bubo.
- Review in 14 days: if no better, refer.



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¹For benzathine benzylpenicillin 2.4MU injection: dissolve **benzathine benzylpenicillin** 2.4MU in 6mL **lidocaine 1%** without epinephrine (adrenaline) and give half the volume into each buttock. ²For ceftriaxone 250mg IM injection: dissolve 250mg in 0.9mL **lidocaine 1%** without epinephrine (adrenaline). ³History of anaphylaxis, urticaria or angioedema. ⁴Advise no alcohol until 24 hours after last dose of metronidazole.